

Medical Evidence Index

Court Participation | Participation Measures | Disability | Fair Trial | Equality Act 2010 | FPR Part 3A | Judicial Review

Re: Mrs Nadia Zahmoul | DOB: 27 April 1969 | NHS No: 705 199 5111

This index covers 14 medical and clinical records spanning April 2013 to April 2026. The 'Neutral Summary' column provides a factual description of each document suitable for inclusion in a court bundle index. The 'Notes & Comments' column identifies the relevance of each document to court participation, participation measures under FPR Part 3A and PD3AA, disability under the Equality Act 2010, fair trial, and Judicial Review grounds. This document is private and confidential and is intended for use in legal proceedings only. It should be read alongside any existing schedule of participation measures, Ground Rules Hearing orders, and the Applicant's skeleton argument.

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1	02 April 2013	Dr Deborah Turner MD Cottonwood Tucson Arizona, USA	Inpatient Psychiatric Evaluation	Psychiatric evaluation following inpatient admission on 31 March 2013 at Cottonwood Tucson, a mental health and substance use disorder centre in Arizona. Records presenting complaint of cocaine dependence in the context of acute psychosocial stressors. Documents extensive trauma history including childhood abuse and reported domestic abuse within marriage. DSM-IV diagnoses: PTSD (309.81) and Cocaine Dependence (304.20). GAF score: 40. Medication: Cymbalta (duloxetine) 120mg. Treatment plan included individual therapy, EMDR and liaison with London-based treating psychiatrist Professor Libby. Electronically authenticated by Dr Turner on 3 April 2013.	EARLIEST FORMAL RECORD OF PTSD DIAGNOSIS. Confirms the Applicant's mental health conditions are chronic, clinically recognised and longstanding, predating all court proceedings by many years. (1) Disability status under Equality Act 2010 s.6 — PTSD with a GAF of 40 plainly has a substantial and long-term adverse effect on normal day-to-day activities. (2) Rebuts any suggestion that participation difficulties are recent, exaggerated or litigation-driven. (3) Establishes the clinical baseline from which all subsequent deterioration must be measured. (4) EMDR recommendation consistent with a severe and complex trauma presentation. (5) Should be read alongside Document 2 (Prof Libby 2022) which traces continuity of treatment from this point.
2	12 July 2022	Professor Gerald Libby FRCP FRCPsych 9 Upper Wimpole Street London W1G 6LJ	Medico-Legal Letter to Solicitor (Withers LLP)	Urgent letter from treating consultant psychiatrist to the Applicant's solicitor (Diana Parker, Withers LLP) during divorce proceedings. Confirms PTSD diagnosis first made at Cottonwood in April 2013. States records from 2011–2013 confirm the Applicant was subjected to abuse and coercive control within her marriage. States that the divorce proceedings and a specific Court Order triggered major PTSD symptoms. Symptoms recorded: stuttering; intermittent mutism and freezing; hypervigilance; extreme anxiety; inability to sleep and eat; inability to self-modulate emotional control; feelings of hopelessness and powerlessness. States the Applicant should not be required to sign documents perceived as threatening.	DIRECTLY ENGAGES PARTICIPATION MEASURES AND FAIR TRIAL. (1) Senior Consultant Psychiatrist (FRCP FRCPsych) identifies specific symptoms — mutism, freezing, hypervigilance — recognised as vulnerabilities under FPR Part 3A and PD3AA. (2) Clinician causally links court proceedings to triggering of those symptoms — evidential foundation for a Ground Rules Hearing. (3) Symptoms prevent effective participation: a party who freezes and becomes mute cannot give or follow evidence on an equal footing. (4) Clinician's opinion that the Applicant should not be compelled to engage with threatening documents is

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					directly relevant to how documents are presented to her in proceedings. (5) Engages Equality Act 2010 s.20–21 reasonable adjustments duty and FPR r.3A.4.
3	21 August 2024	Professor Gerald Libby FRCP FRCPsych 9 Upper Wimpole Street London W1G 6LJ	Welfare Letter – To Whom It May Concern	Urgent welfare letter from treating consultant psychiatrist. Confirms the Applicant has been known and treated since 2011 and has mental capacity. Records grave financial hardship resulting in lack of stable housing and probable malnutrition, evidenced by recent test results showing anaemia and newly diagnosed autoimmune Sjogren's Syndrome. States the Applicant must have legal representation but financial difficulties impact this. States that participation measures which were in place for earlier hearings must now be reinstated. States the combination of unrepresented proceedings and financial hardship places the Applicant under severe pressure and risk.	EXPLICITLY REFERENCES PARTICIPATION MEASURES AND CALLS FOR REINSTATEMENT. (1) Clinician confirms participation measures were previously in place — establishing the court had already accepted the Applicant's vulnerability on a prior occasion. (2) Clinician states those measures must be reinstated — direct medical evidence in support of the relief sought. (3) Confirms removal of participation measures is causing harm — relevant to lawfulness of any decision to withdraw measures. (4) Long-term treating clinician (since 2011) — significant clinical weight. (5) Confirms capacity is not in issue — this is a participation question, not a capacity question. (6) Physical health conditions add further grounds of disability under Equality Act 2010 s.6. (7) Financial hardship preventing legal representation relevant to Article 6 ECHR and equality of arms.
4	14 September 2024	Dr Clare Thompson MBBS BSc nMRCGP DCH DRCOG LMCC Courtfield Private Practice London SW5 0NL	GP Letter – Private & Confidential	Confidential GP letter confirming diagnoses of Sjogren's Syndrome and new anaemia. Records that due to financial constraints the Applicant has been unable to eat and drink adequately and has no housing. Notes the Applicant is responsible for two dependants: a 19-year-old child with special needs and a 17-year-old minor. Dr Thompson states she is concerned the Applicant has insufficient resources to meet basic needs. States intention to refer the case to Social Services to help the Applicant access basic amenities and to safeguard her and her children.	CORROBORATES WELFARE CONCERNS AND ESTABLISHES PHYSICAL DISABILITY. (1) Confirms physical health diagnoses (Sjogren's Syndrome, anaemia) alongside pre-existing mental health conditions — disability on multiple grounds under Equality Act 2010 s.6. (2) GP's safeguarding referral to Social Services evidences the level of clinical concern. (3) Inability to meet basic needs directly relevant to proportionality of continuing proceedings without participation support and legal representation. (4) Evidences material impact of proceedings on the Applicant's physical welfare — relevant to human rights proportionality in Judicial Review. (5) Confirms responsibility for a child with special needs — relevant to the welfare jurisdiction.
5	04 March 2025	James Pullen Trainee ACP / RMN	NHS Crisis Team Assessment Report	Comprehensive NHS crisis assessment at the Applicant's home in Cheltenham (NHS No: 705 199	DOCUMENTS ACUTE CRISIS DURING PROCEEDINGS — FORMAL NHS VULNERABILITY RATING.

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		Gloucestershire Health and Care NHS Foundation Trust Crisis Resolution & Home Treatment Team		5111). Applicant taken to A&E by paramedics; received oral lorazepam and IM haloperidol in A&E. MHLT found no overt evidence of psychosis; presentation attributed to acute stress reaction; referred to CRHTT for further assessment. Mental State Examination: kempt, good eye contact, initially anxious, frequently tearful; no formal thought disorder; no perceptual abnormalities; no suicidal ideation; insight intact. Risk assessment: Vulnerability MEDIUM; LOW in all other areas. Plan: referral to CRHTT; consent for information sharing with Professor Libby and daughter's social worker.	(1) Formal clinical risk assessment rates the Applicant as MEDIUM vulnerability — directly supporting vulnerable party status under FPR PD3AA para 4. (2) Acute crisis occurred during active court proceedings — consistent with causal link between proceedings and deterioration identified by Professor Libby. (3) Absence of psychosis and intact insight confirm the Applicant is not delusional — her difficulties are participatory, not capacity-based. (4) Administration of IM haloperidol without psychosis finding illustrates acute severity of distress and risk of unmitigated court stress. (5) MSE findings (tearful, anxious) directly relevant to how the Applicant should be questioned at any Ground Rules Hearing.
6	26 March 2025	Dr Leigh Poyser MBBS MRCPsych Consultant Psychiatrist South K&C CRHTT CNWL NHS Foundation Trust	Crisis Team Letter Addressed Directly to the Court	Letter from NHS Consultant Psychiatrist addressed to the Court ahead of the hearing on 31 March 2025. Confirms the Applicant is under the care of the South K&C CRHTT due to a mental health crisis. States she is experiencing significant agitation and poor sleep exacerbated by the stress of the court case. Notes specific difficulty processing aspects of the proceedings, particularly evidence she perceives as untrue. States the team is providing intensive support to enable attendance at the hearing. Notes a pending autism assessment as clinically pertinent. Requests the court enquire whether an advocacy service or other appropriate support can be made available.	STRONGEST SINGLE DOCUMENT FOR PARTICIPATION MEASURES — NHS CONSULTANT PSYCHIATRIST WRITING DIRECTLY TO COURT. (1) Consultant Psychiatrist (MRCPsych) writes directly to court confirming current mental health crisis causally linked to proceedings. (2) Identifies specific cognitive and processing difficulties engaged by proceedings, anticipating the autism diagnosis (Document 7). (3) Explicitly requests the court make advocacy and participation support available — a clinical recommendation from the NHS, not merely a legal submission. (4) Flags pending autism assessment as relevant to participation before formal diagnosis. (5) Squarely satisfies the evidentiary threshold for a Ground Rules Hearing under FPR PD3AA. (6) Relevant to FPR r.3A.4 (duty to consider vulnerability) and r.3A.7 (participation directions). (7) If participation measures were not put in place following this letter, or were subsequently withdrawn, this letter is central evidence for a Judicial Review of that decision.
7	20 June 2025	Dr Catherine Cheung Principal Clinical Psychologist	Autism Assessment Outcome Letter	Formal outcome letter from CLAAS confirming diagnosis of Autism Spectrum Condition (ASC) under DSM-5 (299.00; ICD-10: F84.0). Assessment	FORMAL NHS DIAGNOSIS OF A PROTECTED CHARACTERISTIC — AUTISM AS DISABILITY UNDER EQUALITY ACT 2010.

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		(HCPC: PYL32865) Sophia Iacovou Assistant Psychologist CLAAS, CLCH NHS Trust		conducted in accordance with NICE Guidance. Confirms the condition is lifelong and neurodevelopmental. Notes significant difficulties in social communication, managing social relationships and information processing. States explicitly that the Applicant can communicate using sophisticated vocabulary which may create a false impression she is managing better than she is — her challenge lies in the pragmatic use of language. States the Applicant is entitled to support and reasonable adjustments to meaningfully participate in court. Confirms a further letter of clinical recommendations for adjustments will follow.	(1) Autism Spectrum Condition satisfies both the 'substantial' and 'long-term' limbs of Equality Act 2010 s.6 without further argument. (2) CLAAS clinicians explicitly state the Applicant is entitled to reasonable adjustments for court participation — directly engaging the Equality Act 2010 s.20 duty. (3) Warning that sophisticated vocabulary may create a false impression of capability is critically important — the court cannot infer from articulacy that no adjustments are required. (4) Challenge lies in pragmatic language use, not vocabulary — relevant to how questions should be framed and answers received in proceedings. (5) Relevant to FPR PD3AA para 5 and the court's obligation not to disadvantage a party by reason of a protected characteristic.
8	01 July 2025	Dr Catherine Cheung Principal Clinical Psychologist (HCPC: PYL32865) Sophia Iacovou Assistant Psychologist CLAAS, CLCH NHS Trust	Full Autism Diagnostic Assessment Report	Comprehensive autism diagnostic report following assessment on 15 May 2025 (NHS No: 705 199 5111). Referral made specifically to inform court participation adjustments. Assessment tools: clinical interview; ADOS-2; RAADS-R questionnaire; professional liaison. Diagnostic outcome: autistic (confirmed). Detailed autism profile recorded including: formal and metaphorical language; conversational turn-taking difficulties; challenges interpreting social cues; preference for direct transparent communication; strong sense of fairness; analytical and rule-based information processing; detail-focused memory; preference for structure and predictability; sensory sensitivities including sensitivity to courtrooms. Co-occurring PTSD noted. Records that previously granted court participation adjustments have been withdrawn and links this to deterioration. Recommended adjustments: alternative communication formats; extra time; support to organise thoughts.	MOST DETAILED CLINICAL DOCUMENT FOR PARTICIPATION MEASURES — GOLD-STANDARD ASSESSMENT SPECIFICALLY COMMISSIONED FOR COURT. (1) ADOS-2 is the internationally validated gold-standard observational tool for autism diagnosis — methodologically robust and NICE-compliant. (2) Referral made specifically to inform court adjustments — the NHS itself identified court participation as the clinical purpose. (3) Previously granted participation measures confirmed to have been withdrawn and causally linked to deterioration — central to any Judicial Review ground. (4) Sensory sensitivity to courtrooms formally documented — relevant to physical hearing arrangements. (5) Specific recommended adjustments map directly onto FPR PD3AA paras 5–7 participation directions. (6) Standard structure of legal proceedings does not align with the Applicant's communication style — without adjustment she cannot participate on an equal footing. (7) Rule-based processing explains difficulty accepting evidence perceived as untrue — an autistic trait, not non-compliance. (8) Autistic traits expressly stated not to correlate with parenting capability.

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9	03 July 2025	Dr Catherine Cheung Service Lead & Principal Clinical Psychologist Sophia Iacovou Assistant Psychologist CLAAS, CLCH NHS Trust	CLAAS Reasonable Adjustments & Court Participation Letter	Clinical letter from CLAAS Service Lead addressed to whom it may concern. Confirms the Applicant's formal autism diagnosis under DSM-5. States that court environments — unfamiliar, involving significant social communication demands and requiring quick information processing under time pressure — are particularly stressful for autistic individuals. Sets out specific autistic traits relevant to legal proceedings: preference for direct honest communication; formal/metaphorical language; difficulty with conversational turn-taking; analytical and rule-based thinking; strong sense of justice; detail-focused memory. Sets out comprehensive recommended reasonable adjustments across communication, information processing, emotional regulation and sensory needs. Notes the Applicant reported significant distress following the recent withdrawal of previously granted court accommodations. Strongly recommends consistent implementation of adjustments throughout the remainder of proceedings.	(9) All services are NHS — not a privately commissioned litigation report. KEY OPERATIONAL DOCUMENT FOR PARTICIPATION DIRECTIONS — SPECIFIC RECOMMENDED ADJUSTMENTS FROM NHS SPECIALIST SERVICE. (1) Written by the Service Lead of CLAAS — the highest clinical authority within that service. (2) Explicitly addressed to the court and intended to support equitable participation — the document to attach to any FPR Part 3A application or Ground Rules Hearing request. (3) Specific adjustments recommended: additional time to express thoughts; clear direct unambiguous language; shorter sentences; no vague or contradictory statements; specific rather than open-ended questions; advance provision of documents; regular breaks; quiet waiting area; video link participation if possible; advocacy support. These map directly onto FPR PD3AA para 5. (4) Expressly confirms autistic traits have no bearing on parenting capacity. (5) Explicitly records distress at withdrawal of previously granted accommodations and strongly recommends reinstatement — directly relevant to any Judicial Review. (6) Clearly states this is a clinical letter, not expert evidence — relevant to admissibility.
10	25 October 2025	DuRoss, John PA-C St. John's Urgent Care Jackson, Wyoming, USA MRN: 2018366	Urgent Care Clinic Note – Sjogren's Syndrome Flare	Urgent care clinic note from St. John's Urgent Care, Jackson, Wyoming, USA. 56-year-old female with confirmed Sjogren's Syndrome by biopsy presents with concern for flare: 4 days of sinus pressure, dryness, fatigue, swollen glands, mouth and eye irritation. Had run out of hydroxychloroquine 200mg daily and sought refill. PHQ-2 depression screen: score 0. Diagnosis: Sjogren's Syndrome flare (ICD: M35.00). Treatment: hydroxychloroquine refilled; prednisone 20mg daily for 5-day burst. Ongoing medication list recorded including fluoxetine, lisdexamfetamine (Vyvanse 70mg), estradiol, progesterone, propranolol, omeprazole, sucralfate and dextroamphetamine-amphetamine.	EVIDENCES ONGOING AND ACTIVE PHYSICAL DISABILITY AND CONTINUED MEDICATION COMPLEXITY. (1) Sjogren's Syndrome diagnosis confirmed by biopsy — the highest level of clinical confirmation available. Strengthens the physical disability ground under Equality Act 2010 s.6. (2) Documents active disease flare requiring urgent medical intervention — relevant to the Applicant's current health status and ability to travel and attend court. (3) Medication list (including Vyvanse, fluoxetine, propranolol and hormone therapy) consistent with a complex picture of neurodevelopmental, psychiatric, autoimmune and hormonal conditions — relevant to the totality of disability.

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					(4) Records the Applicant is registered with a US primary care physician — relevant to any question about travel, attendance and timing of hearings. (5) PHQ-2 score of 0 should be read in context: it is a brief two-question depression screen only and does not screen for PTSD, autism or anxiety disorders.
11	25 November 2025	Dr Bruce Hayse MD P.O. Box 1884 Jackson Hole Wyoming 83001, USA Tel: (307) 733-6700	Medical Letter to the Court – Treating Physician USA	Letter from treating physician in Jackson Hole, Wyoming, addressed directly to the Court. Confirms the Applicant is under his care and is currently clinically vulnerable due to prolonged uncertainty around ongoing legal proceedings in the United Kingdom. Records recent mental health crises, emotional breakdowns, episodes of acute distress, functional impairment and heightened anxiety. Confirms diagnoses of ASC, PTSD and Sjogren's Syndrome. States it is important the court process considers her communication and information-processing needs. Notes that difficulties with the opacity and ambiguity of the legal process previously caused severe distress resulting in two hospitalisations in London in March and June 2025. Endorses the NHS Autism Assessment Report (1 July 2025) and the CLAAS Reasonable Adjustments letter (3 July 2025). States the Applicant is fit to travel to the UK for a court hearing provided it is scheduled expeditiously, reasonable adjustments are made, and she is permitted appropriate support. States any prolonged delay or remote determination would be medically unsafe.	INDEPENDENT CORROBORATION FROM TREATING PHYSICIAN IN USA — COURT-ADDRESSED LETTER DIRECTLY RELEVANT TO PARTICIPATION MEASURES AND HEARING FORMAT. (1) Licensed US physician writes directly to the UK court confirming current clinical vulnerability and need for participation measures — independent corroboration from outside the NHS. (2) Explicitly identifies that court opacity, ambiguity and delay are themselves medically harmful — directly relevant to arguments about administrative silence, remote hearings and paper determinations. (3) States that paper-based decisions, remote hearings, administrative silence and procedural lack of clarity would be 'medically unsafe' — a clinician's direct opinion on hearing format. (4) Clinically indicates an oral in-person hearing scheduled as soon as reasonably possible — a medical recommendation about the form of the hearing. (5) Confirms fitness to travel to the UK — conditional on expeditious scheduling, adjustments and appropriate support. (6) Records two hospitalisations for mental health crises in London in March and June 2025 — corroborating Documents 5 and 6. (7) Explicitly endorses and cross-references the NHS CLAAS reports (Documents 8 and 9). (8) Relevant to FPR r.3A.4, FPR PD3AA, Equality Act 2010 s.20–21 and Article 6 ECHR.
12	28 November 2025	Stoecklein, Holbrook MD St. John's Medical Center Jackson, WY 83001, USA MRN: 2018366	Emergency Department Note – Acute Flank Pain / Kidney Stone	Emergency department note from St. John's Medical Center, Jackson, Wyoming. 56-year-old female presents via EMS with sharp right-sided flank pain of 5–6 hours duration. History of 5 prior kidney stones. EMS administered diazepam and fentanyl prior to arrival. In ED: received ketorolac and droperidol IV; CT abdomen/pelvis with contrast ordered. Labs: hematuria on urinalysis; RBC low;	EVIDENCES CONTINUING PHYSICAL HEALTH DETERIORATION AND ACUTE MEDICAL EMERGENCIES DURING PROCEEDINGS. (1) Emergency hospital attendance on 28 November 2025 — further acute medical event consistent with the pattern

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				lipase mildly elevated. Psychiatric finding on physical examination: extremely anxious, psychomotor agitation. Provisional diagnosis: ureterolithiasis most likely. Discharge diagnosis: hematuria; abdominal pain — cause unknown. Discharged with instructions to return if symptoms worsen.	of physical and mental health deterioration evidenced across this bundle. (2) Psychiatric finding — 'extremely anxious, psychomotor agitation' — recorded by a US emergency physician with no prior knowledge of the legal proceedings, providing independent corroboration. (3) Low RBC on blood results consistent with previously documented anaemia (Documents 3 and 4) — corroborating that anaemia is an ongoing condition. (4) History of 5 previous kidney stones consistent with the chronic and complex medical history evidenced throughout this bundle. (5) Weight recorded at 48.5kg (BMI 19.56) — a further data point relevant to the previously documented concerns about malnutrition (Documents 3 and 4). (6) Should be read alongside Document 13 (discharge summary) as a paired record.
13	10 December 2025	Reyes, Barron MD St. John's Medical Center Jackson, WY 83001, USA MRN: 2018366	Emergency Department Note – Mental Health Crisis / Anxiety Crisis	Emergency department note from St. John's Medical Center, Jackson, Wyoming. Chief complaint recorded in the patient's own words: 'ANXIETY CRISIS. I HAVE AUTISM AND PTSD.' 56-year-old female with reported history of PTSD, autism and severe anxiety presents reporting acute stress arising from legal proceedings and the need to return to London for a lawsuit. Reports running out of benzodiazepine medication. Reports prior hospitalisations for emotional distress. Psychiatric examination: very anxious, hyperventilating, tearful, constantly moving around. Treatment: droperidol IV push; lorazepam (Ativan) oral; acetaminophen oral. Patient stable following treatment; discharged home. Formal diagnosis: (1) Emotional crisis, acute reaction to stress (F43.0); (2) Medication refill.	MOST RECENT DOCUMENT — EMERGENCY HOSPITALISATION DIRECTLY CAUSED BY LEGAL PROCEEDINGS. HIGHLY SIGNIFICANT FOR JUDICIAL REVIEW. (1) Chief complaint, recorded in the patient's own words, directly and explicitly links the acute medical emergency to the legal proceedings and the requirement to return to London — a contemporaneous, unmediated statement in a US emergency hospital record. (2) Formal diagnosis of 'Emotional crisis, acute reaction to stress (F43.0)' confirms the clinical presentation was caused by acute stress — corroborating the causal link between the proceedings and the Applicant's deteriorating health. (3) This is the third US emergency medical attendance documented in this bundle (Documents 10, 12, 13) within a 7-week period (25 October to 10 December 2025) — evidencing a pattern of acute and escalating medical crises. (4) Self-identification as having 'autism and PTSD' recorded in an independent US hospital record provides further independent corroboration of the NHS diagnoses. (5) Psychiatrist's examination findings (hyperventilating, constantly moving, tearful) directly consistent with the autistic and PTSD presentations documented by NHS clinicians (Documents 5–9).

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					(6) Weight again recorded at 48.5kg — consistent with Document 12 and ongoing malnutrition concerns. (7) This document alone provides strong grounds for urgent case management directions, reinstatement of participation measures, and expeditious scheduling of an oral in-person hearing as recommended by Dr Hayse (Document 11).
14	17 April 2026	Dr Omar Shoukry Speciality Doctor in General Adult Psychiatry SK&C Community Mental Health Team South Kensington & Chelsea Mental Health Unit 1 Nightingale Place Chelsea SW10 9NG Tel: 0207 685 6248	NHS CMHT Letter Addressed Directly to the Court	Letter from NHS Speciality Doctor in General Adult Psychiatry, South Kensington & Chelsea Community Mental Health Team (CMHT), CNWL NHS Foundation Trust, addressed directly to the Court. Confirms the Applicant has been under the care of the CMHT since December 2025, following a mental health crisis in early 2025. Confirms ongoing symptoms of significant psychomotor agitation and poor sleep, exacerbated by the stress of the court case. Records specific difficulty processing aspects of the proceedings, particularly evidence the Applicant perceives as untrue. Confirms the team is providing intensive support with the aim of ensuring the Applicant is well enough to attend court hearings. Confirms the formal autism spectrum disorder diagnosis (June 2025) and the co-morbid PTSD diagnosis. Records that these factors may influence how the Applicant perceives and processes information during court proceedings. Requests the court enquire whether an advocacy service or other appropriate support can be made available to assist the Applicant in hearing and giving evidence at the hearing.	CONTINUING NHS CARE — CMHT ENGAGEMENT FROM DECEMBER 2025 TO PRESENT — DIRECT REQUEST TO COURT FOR ADVOCACY AND PARTICIPATION SUPPORT. (1) NHS CMHT Speciality Doctor in General Adult Psychiatry writes directly to court confirming continuing clinical care and ongoing mental health needs causally linked to the proceedings. (2) Confirms the Applicant has been under CMHT care since December 2025 — the most recent documented NHS engagement and demonstrates that the harm identified in Documents 5–13 is continuing, not historic. (3) Reiterates the specific cognitive and processing difficulties engaged by proceedings — consistent with the autism and PTSD diagnoses confirmed at Documents 7, 8 and 9. (4) Explicitly requests the court make advocacy and participation support available — a clinical recommendation from the NHS, made at the most senior community psychiatric level. (5) Squarely satisfies the evidentiary threshold for participation directions under FPR PD3AA. (6) Relevant to FPR r.3A.4 (duty to consider vulnerability) and r.3A.7 (participation directions). (7) Evidences that as at April 2026, the position remains as the CLAAS clinicians described in July 2025 — without reasonable adjustments, the Applicant cannot participate equitably in court proceedings. (8) Together with Document 6 (Dr Poyser, 26 March 2025) and Document 11 (Dr Hayse, 25 November 2025), this letter forms the third sequential direct-to-court clinical letter requesting participation measures. Three separate clinicians, three separate clinical services, three separate dates over a 13-month period, all making the same clinical request. (9) The continued absence of any judicial act in response to these direct-to-court clinical letters is itself central evidence for any subsequent Judicial Review, MPS criminal referral, or other proceeding.

NOTE: All documents listed are either NHS records, records from treating clinicians, or US medical records. None are privately commissioned medico-legal reports obtained for the purposes of litigation. Further medical documents may be added to this index as they become available.